

Individual Case Safety Report (ICSR)

Task 5 - structured safety assessment and case documentation

CASE REFERENCE	REPORT TYPE	CASE STATUS
ACADEMIC-ICSR-2026-002	Fictional spontaneous case simulation	Initial case report
EVENT DATE	REPORT DATE	SERIOUSNESS
26 Sep 2026	27 Sep 2026	Non-serious

1. Patient and case information

FIELD	CASE INFORMATION
Patient	R.K. - fictional initials only
Age / sex / weight	28 years / male / 70 kg
Location	Vadodara, Gujarat, India
Relevant history	No significant medical history reported; no known drug allergy reported.
Indication	Suspected infection (as recorded in the source case; specific diagnosis not supplied).
Medical confirmation	No laboratory or other diagnostic test data provided; not applicable in the source case.

2. Suspected reaction

FIELD	RECORDED INFORMATION
Reaction	Urticaria (hives), generalized itching, and mild facial swelling.
Onset / latency	26 Sep 2026; approximately 30 minutes after the suspected medicine was taken.
Course / outcome	Amoxicillin withdrawn; supportive treatment provided; symptoms subsided and patient recovered. Specific treatment and recovery date were not provided.
Seriousness	Non-serious in the source case: no hospitalization, life-threatening event, disability, or congenital anomaly reported.
Relevant clinical impact	Mild facial swelling was reported with urticaria and itching. No airway compromise or other serious criterion was reported in the source case.

3. Suspected and concomitant medicines

MEDICINE	DOSE / ROUTE	USE / ACTION
Amoxicillin - suspected	500 mg orally; single dose	For suspected infection. Stopped after reaction. Start date recorded as 26 Sep 2026.
Paracetamol - concomitant	500 mg orally	For fever / pain. Continued; no reaction noted.

Case validity: the source case contains the four core elements for a reportable safety case: an identifiable (fictional) patient, identifiable (fictional) reporter, suspected medicine, and adverse reaction.

Assessment, Reporting & Review

Continuation of case reference ACADEMIC-ICSR-2026-002

4. Causality assessment

FACTOR	CASE EVIDENCE	ASSESSMENT
Temporal relationship	Reaction began about 30 minutes after a single oral amoxicillin dose.	Supports association; rapid onset is temporally compatible with an acute hypersensitivity-type event.
Dechallenge	Amoxicillin was stopped; symptoms subsided and patient recovered.	Positive dechallenge reported.
Alternative causes / concomitant medicine	Paracetamol was also taken and continued, with no reaction noted. Other causes are not described.	Amoxicillin is the more temporally suspect medicine in the source narrative; alternative causes are not fully evaluated.
Rechallenge	Not performed.	No rechallenge evidence.
Overall causality	Probable (as recorded in the source case).	Temporal association and improvement after withdrawal support a probable relationship. Evidence remains limited by the single-case narrative and unassessed alternatives.

5. Pharmacovigilance classification

CLASSIFICATION	CASE CONCLUSION
Event type	Suspected Type B ADR - unpredictable / not dose-dependent, as designated in the Task 2 academic case.
Seriousness	Non-serious, based on the reported absence of the listed serious outcomes.
Expectedness	Not independently assessed. No product information / reference safety information was included in the supplied case materials.
Outcome	Recovered.
Overall case conclusion	Probable amoxicillin-associated acute hypersensitivity-type reaction (urticaria, generalized itching, mild facial swelling); non-serious; recovered.

6. Reporter and report administration

FIELD	RECORDED INFORMATION
Reporter	Dr. A. Patel (fictional), Medical / Pharmacy Professional
Professional address	Academic Case Exercise, Vadodara, Gujarat 390001
Contact	academic.case@example.com +91-90000-00000 (fictional details)
Report date	27 Sep 2026
Report status	Initial academic case report; no follow-up report number or regulator submission recorded.

7. Summary and data limitations

A 28-year-old fictional male developed urticaria, generalized itching, and mild facial swelling approximately 30 minutes after a single 500 mg oral dose of amoxicillin. The medicine was stopped, supportive treatment was provided, and the patient recovered. Paracetamol was continued without a reported reaction. The source assessment is probable causality and non-serious outcome. Exact recovery date, treatment details, indication diagnosis, product batch, and reference safety information were not supplied.

Confidentiality: This is a fictional educational exercise. Patient initials, reporter identity, contact information, and clinical details are reproduced solely as fictional case material from the supplied Task 2 PDF. This document is not a real patient record or regulatory submission.